Name: Mr. Jaikey Chauhan 38 yrs.

Reffered by: Rajdhani Hospital

Collection Date and Time 28-07-2026 06:21 PM

Reporting Date and Time 28-07-2026 06:35 PM

Booked tests CBC, LFT, Typhi Dot IgG, Typhi Dot IgM

TOTAL CARE & CONTROL TCC diagnostics

PATHOLOGY LAB

TEST REPORT

Test Name	Patient Values	Units	Normal values
Complete Blood Count- CBC			
	Hemoglobin		
Hemoglobin - EDTA Blood *Non-Cyanide Haem Method*	14.2 gm/dl		Males: 13.0-17.0 g/dL Females: 12.0-15.0 g/dL Birth:14.0-22.0 g/dL 1-7 days: 13.5-21.5 g/dL 0-1 month: 11.6-16.5 g/dL 1month-1year: 11.1-14.1 g/dL 2-6 years: 11.0-14.0 g/dL 2-6 Years: 5-15 ^3/microL
Hematocrit (HCT) - EDTA Blood *RBC Pulse height detection*	42.0		37-49
	Cell Counts		
Total Leukocyte Count - EDTA Blood *Impedance*	3.6 x 10^3/microL		4-10 x 10^3/microL Birth:10-26 ^3/microL 1-7 days: 6-22 ^3/microL 0-1 month: 5-19 ^3/microL 1month-1year: 6-16 ^3/microL 2-6 Years: 5-15 ^3/microL
Red Blood Cell count (RBC) - EDTA Blood *Impedance*	5.00 x 10^6/ microL		Males: 4.5-5.5 x 10^6 Females: 3.8-4.8 x 10^6/ microL
Platelet Count - EDTA Blood *Impedance*	1.73 Lacs/ microL		1.5 - 4.1 Lacs / microL
	Differential Counts		
Neutrophils - EDTA Blood *Impedance/Microscopy*	56 %		40-75%
Lymphocytes - EDTA Blood *Impedance/Microscopy*	36 %		25-40%
Eosinophils - EDTA Blood *Impedance/Microscopy*	04 %		0-6%
Basophils - EDTA Blood *Impedance/Microscopy*	00 %		0-1%
Monocytes - EDTA Blood *Impedance/Microscopy*	04 %		2-10%
	Hematology Other Values		
Mean Corp. Volume (MCV) - EDTA Blood *Calculated*	84.0 fl		83-101 fl
Mean Corp. Hemoglobin (MCH) - EDTA Blood *Calculated*	28.5 pg		27-32 pg

TOTAL CARE & CONTROL TCC diagnostics

PATHOLOGY LAB

TEST REPORT

Test Name	Patient Values	Units	Normal values
Mean Corp. Hemoglobin Conc. (MCHC) - EDTA Blood *Calculated*	33.9 gm/dl		31.5-34.5 gm/dl

Immuno Assay

Typhi Dot IgG - Serum *Typhi Dot IgG*	Negative		Negative
		Others	
Typhi Dot IgM - Serum *Typhi Dot IgM*	Negative		Negative

Typhi Dot IgM test detects IgM type antibodies to Salmonella Typhi which are formed due to body's response to Typhoid (enteric fever) infection. The conventional Widal test detects antibodies to S.Typhi only after 14 days of onset of fever and infection and may remain positive even for a long period after convalescence, hence delaying diagnosis and increasing chances of false positives..But antibodies predominantly IgM are markers of recent Typhoid infection.
Typhoid is a serious illness if untreated may lead to complications of Brain, Lungs, Liver and other organs, hence early diagnosis is important for treatment.
Detectable IgM response

Onset of fever	Percent Positive
4-6 days	42.5
6-9 days	92.9
> 9 days	100

LFT (Liver Function Tests)

		Biochemistry	
Billirubin Total - Serum *Diazo Method End Point-*	0.54 mg/dl		Adults: 0-1.20 mg/dl; Cord < 2mg/dl; Newborns, premature: 0-1d 1.0-8.0 mg/dl; 1-2 d 6.0-12.0 mg/dl; 3-5d 10.0-14.0; Newborns Full Term: 0-1d 2.0-6.0 mg/dl, 1-2d 6.0-10.0 mg/dl, 3-5d 4.0-8.0 mg/dl

Billirubin is the breakdown product of heamoglobin, This Billuribin is water Insoluble and is known as Indirect or unconjugated form. Billirubin is changed into water soluble form by the Liver and excreted through Bile into the intestines. Total Billirubin is elevated in obstructive conditions of Bile duct, Hepatitis, Cirrhosis and several other Disorders

Billirubin Direct - Serum *Diazo Method End Point*	0.18 mg/dl	Less than 0.2 mg/dl

Also called conjugated Bilirubin

Billirubin Indirect - Serum *Calculated*	0.36 mg/dl	Less than 0.8 mg/dl

Indirect Billirubin may be elevated due to pre Hepatic causes like Hemolysis, or Severe Liver Diseases resulting in impaired entry or transportation in Liver. It is of special importance in Neonates as may cause damage to the Brain

Total Protein - Serum *Biuret method- End point*	7.6 g/dl	6-8.3 g/dl

Total Protein may be increased due to Dehydration, Chronic Liver Disease, Multiple Myeloma etc, it decreases in Chronic Renal disease and Terminal Liver failure and other causes.

RegistrationID: **69141**
UID **80527**

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TEST REPORT

Test Name	Patient Values	Units	Normal values
Albumin - Serum *BCG Dye method-End point*	4.0 g/dl		0-4d 2.8-4.4 g/dl; 4d-14y 3.8-5.4g/dl; 14-18y 3.2-4.5g/dl; 20-60y 3.2-5.2 g/dl;60-90y 3.2-4.6g/dl;3.2-4.6 g/dl

Protein synthesized by Liver, Increases due to Dehydration, water loss, decreased levels are seen in Kidney skin or Intestine disorders, , Dietary deficiency, Liver Disease, Fever and Diabetes etc..

Globulin - Serum *Globulin*	4 mg/dl		2.8-3.3

An elevated serum Globulins (>4 g/dL) suggests some sort of systemic inflammation. It can be noted in active Connective Tissue Disorders, Inflammatory Bowel Disease, chronic infections, paraproteinemias, etc. It warrants further testing, especially:

Serum Protein Electrophoresis (SPEP), to rule-out Multiple Myeloma
HIV test

Enzymes

SGPT (ALT) - Serum *IFCC-without Pyridoxal Phosphate*	36.1 IU/L		At 37 deg Celcius: Men upto 45 U/L; Women upto 34 U/L

Increased levels of SGPT are a result of injury to Liver as in Hepatitis, Cirhossis or Obstructive jaundice.

SGOT (AST) - Serum *IFCC-without Pyridoxal Phosphat*	33.2 IU/L		At 37 deg Celcius Men upto 35 U/L; Women upto 31 U/L

Increased levels of SGOT are observed in patients with Liver Disease, Cholecystitis, Myocardial Infarction, Kidney Diseases etc

ALP (Alkaline Phosphatase) - Serum *PNPP-AMP-*	91.4 IU/L		Adults Male: 42 IU/L -141 IU/L ; Adults Female : 20 IU/L - 119 IU/L.

Is present in high concentrations in Liver, bone Placenta, intestines and certain tumours- Is normally raised in Pregnancy and growth period of children; may be increased abnormally due to Liver, Bone Disease Congestive Heart failure, Hodgkins Disease etc…

Others

A:G (Albumin:Globulin) Ratio - Serum *A:G (Albumin:Globulin) Ratio*	1		1.2:1 - 2:1

----------End of Report----------

** Test Not in NABL Scope
*** Outsourced Tests:**

Test Performed by **Technician** *Reporting Doctor*

1. All test reports are for the interpretation by the treating doctor only and should be correlated with the clinical status and other investigations. Isolated Lab reports do not absolutely confirm or rule out individual disease.2. All investigations have their limitations which are imposed by the sensitivity and specificity of the individual assays. 3. Results pertain to the specimens received; Identity of the individuals is not verified.4. If the results of the tests are alarming or unexpected doctors are advised to contact the centre immediately for Confirmatory tests or other needful actions.

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